

MP-214: Lumbar Spinal Fusion

Policy Number: MP-214 | Effective Date: 01/01/2026 | Category: Surgery - Musculoskeletal / Spine | Approved by: Medical Policy Committee

1.0 SCOPE

- 1.1 This policy applies to prior authorization review of elective lumbar spinal fusion procedures, including posterior lumbar interbody fusion (PLIF), transforaminal lumbar interbody fusion (TLIF), and posterolateral fusion, for members of Lakeshore Mutual Health commercial plans.
- 1.2 This policy does not apply to fusion performed for acute fracture, tumor, or infection, which are reviewed under separate criteria.
- 1.3 Coverage is subject to the terms of the member's benefit plan. Where benefit plan language conflicts with this policy, the benefit plan governs.

2.0 DEFINITIONS

- 2.1 **Conservative care:** Non-operative treatment directed at the lumbar condition, which may include supervised physical therapy, home exercise, activity modification, oral medications, and interventional pain management (e.g., epidural steroid injection).
- 2.2 **Failed conservative care:** Persistent pain or functional limitation despite an adequate trial of conservative care, as documented in the medical record.
- 2.3 **Spondylolisthesis:** Anterior or posterior translation of one vertebral body relative to the adjacent vertebra, graded I through IV.
- 2.4 **Instability:** Abnormal segmental motion, generally demonstrated on dynamic (flexion-extension) imaging.
- 2.5 **Progressive neurological deficit:** Objective motor weakness, reflex change, or sensory loss that worsens over serial examinations.
- 2.6 **Active nicotine use:** Use of any tobacco or nicotine-containing product, including cigarettes, cigars, vaping products, and nicotine replacement products.

3.0 MEDICAL NECESSITY CRITERIA

- 3.1 Lumbar spinal fusion is considered medically necessary when **all** of criteria (a) and (b) are met and exclusion criterion (c) does not apply:
 - (a) **Imaging-confirmed spondylolisthesis with instability OR progressive neurological deficit**, with clinical findings that correlate with the imaged level.
 - (b) **At least 3 months of failed conservative care, including physical therapy.**
- 3.2 Requests that do not meet criteria (a) and (b) will be reviewed by a physician reviewer and may be denied as not medically necessary.

3.0 MEDICAL NECESSITY CRITERIA (continued)

3.3 Exclusion criterion:

- (c) **Exclusion:** Active nicotine use, unless documented abstinence for **at least 6 weeks before surgery**. Abstinence should be documented in the treating surgeon's record and may be supported by cotinine testing.

4.0 REQUIRED DOCUMENTATION

4.1 Requests for prior authorization must include the following:

- 4.1.1 Operative plan identifying the specific level(s) to be fused and the procedure type.
- 4.1.2 Advanced imaging report (MRI or CT) obtained within the past 12 months, with the radiologist's report.
- 4.1.3 Physical therapy records showing dates of service, number of visits, interventions, and outcome.
- 4.1.4 Documentation of other conservative measures attempted, including medications and any interventional pain management, with dates and response.
- 4.1.5 Neurological examination findings, including motor strength grading.
- 4.1.6 Documentation of nicotine use status and, if applicable, date of cessation.
- 4.1.7 Planned date of surgery and facility.

5.0 CODING

- 5.1** CPT 22630: Arthrodesis, posterior interbody technique, including laminectomy and/or discectomy to prepare interspace (other than for decompression), single interspace, lumbar.

6.0 REVISION HISTORY

Date	Change
01/01/2026	conservative care requirement revised from 6 months to 3 months.
01/01/2025	Annual review. Nicotine exclusion language clarified to include vaping products.
01/01/2024	Annual review. Required documentation list updated.

7.0 DISCLAIMER

- 7.1** Medical policies are used to guide coverage decisions and do not constitute medical advice. Treating providers are solely responsible for medical care.